On Paper and in Practice: A Short Guide to Comparing Dad's Care Plan With Dad's Week

Why a plan and a real week can read differently, and how to look at both without blaming anyone.

Dad has a care plan in a folder and an agency schedule on the fridge. He also has an actual week, which your sister knows better than anyone. Lately you've had the sense that the two don't quite match.

This guide explains what a care plan records and what it doesn't, and how to set the plan beside the week one need at a time. It's an explanation, not a judgment of Dad's care or of anyone who gives it.

A plan records what was arranged

The Centers for Disease Control and Prevention describes a care plan as a form that summarizes a person's health conditions, specific care needs and current treatments. Where a Medicare-certified home health agency is involved, Medicare says its plan of care lists what services are needed, which health care professionals should give them, and when and how often. [1] [2]

Both describe what is meant to happen. Neither is a record of last Tuesday. That isn't a flaw in the plan. It's a different kind of paper.

A plan is also written at one point in time. The CDC suggests updating a care plan every year, or more often if there is a change in health or medicines, and Medicare says a home health plan of care is reviewed as often as necessary, but at least once every 60 days. A week can shift between one look and the next. [1] [2]

- Work from this week: what you saw, what Dad told you, what your sister did.
- Beside each service on the plan, write “seen” if someone you can name watched it happen, and “scheduled” if you're going on the paperwork.
- “Scheduled” isn't bad news. It means nobody in the family has looked yet.

Three accounts, and where each one came from

There are three accounts of Dad's week: his own, the family's and the professionals'. The National Institute on Aging says the best way to know what someone needs is to ask them directly. The CDC says to begin with the person and, if he can't give all the information, to talk to others who regularly interact with him. [3] [1]

Write each thing down with where it came from and the date: “Dad said, Thursday.” “The agency's schedule, printed in September.” “My sister, on the phone Sunday.” Keep the “don't knows” on the page too.

One caution. If three relatives tell you the aide comes on Mondays, and all three heard it from your sister, that is one account, repeated. It may well be right. It's still one source.

- Ask Dad before you call around. The National Institute on Aging describes contacting the people who see a parent regularly as something done with the person's permission.

Needs on one side, help on the other

To name needs plainly, borrow a public vocabulary. The Administration for Community Living lists the basic personal tasks (bathing, dressing, using the toilet, moving to or from a bed or chair, caring for incontinence, eating) and the everyday ones (housework, managing money, taking medication, preparing and cleaning up after meals, shopping, using the telephone, caring for pets, responding to emergency alerts). They are words for a list. They aren't a scale. [4]

Then the help, all of it. The same agency's page on who provides care says about 80 percent of care at home is provided by unpaid caregivers, so your sister's week belongs on the page beside the agency's. Ask her what she does, day by day. The National Institute on Aging notes that a primary caregiver may be hesitant to ask for help or a break, so ask her; don't wait for her to bring it up. [5] [6]

Leave room for the part of the week that isn't a task. The Family Caregiver Alliance lists emotional care (companionship, meaningful activities, conversation) as an area of help alongside personal care, household care and health care. [7]

- For each need, one line: who covers it, and how you know.
- A need with no name beside it is a blank to ask about. It isn't a verdict on Dad's safety or on anyone's effort.
- If the same task shows up on the plan and in your sister's week, write down both. It's a question to ask, not a conclusion.

Writing a gap down so it can be followed up

How a gap is written decides what happens to it. “The agency never shows up” starts an argument. “The schedule lists a Wednesday visit, and nobody in the family saw one the last two Wednesdays” can be checked.

The National Institute on Aging gives the same advice for conversation: mention your worry without sounding critical. For families working with a home attendant, the Family Caregiver Alliance suggests writing out what is not working and coming together on solutions. [3] [7]

Then sort the open lines by who can answer them. Some belong to the family. Anything about Dad's health, his medications or what a gap means for him belongs to his clinicians; write those as questions and leave the answers blank. Where a Medicare-certified home health agency is involved, Medicare's booklet says to talk to both the provider and the home health team if you have a question about the care or feel needs aren't being met. [2]

What this means for your family

- A plan and a week are two different things. Setting them side by side is ordinary housekeeping, not an accusation.
- Every line on your page is one of three things: confirmed, not sure yet, or not covered by anyone you can name.
- “Not sure yet” is a real answer. It goes on the page with the name of the person who could tell you.
- Your sister's work is part of the arrangement. Write it down, and ask her how she's holding up.
- What the gaps mean, and what should be done about them, isn't the family's call alone. That goes to Dad's clinicians and to your care consultant, with your page in front of them.

What this guide doesn't do

It doesn't say whether Dad's care is working, whether any service has fallen short, or what should change. The definitions, the lists and the review intervals come from the sources below. The “seen or scheduled” habit and the advice about what to write down are ours.

Ready for the next step?

You know now why the plan and the week can read differently, and how to set one beside the other without blaming anyone. The care interview does that comparing against a complete picture: it asks about each area of Dad's daily life in turn and who covers it, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Centers for Disease Control and Prevention, Steps for Creating and Maintaining a Care Plan
2. Centers for Medicare & Medicaid Services, Medicare & Home Health Care (CMS Product No. 10969)
3. National Institute on Aging, Does an Older Adult in Your Life Need Help?
4. Administration for Community Living, What is Long-Term Care?
5. Administration for Community Living, Who Will Provide Your Care?
6. National Institute on Aging, Sharing Caregiving Responsibilities
7. Family Caregiver Alliance, Hiring In-Home Help

From the Family Care Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.